

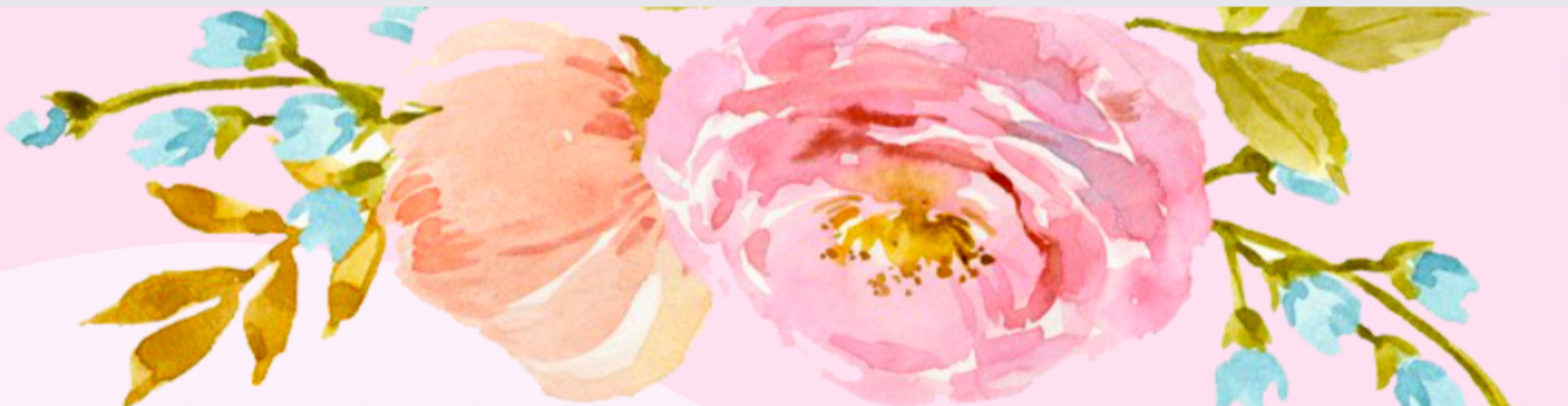


Nclexreview Etsy - nclex

Nclex cram sheet simple nursing (Essential School of Nursing)



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Thank You

For Your Purchase Of The
NCLEX REVIEW

My mission was simple. I wanted to create beautiful and uplifting content that made some of the most complex concepts easier to understand.

The world needs more nurses!



I would be so grateful if you get a chance to leave a review and be sure to follow for more inspiration, ideas, and advice to help you in your present or future nursing career.

XOXO Amanda from Beautiful Nursing

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B e a u t i f u l N u r s i n g

NCLEX REVIEW

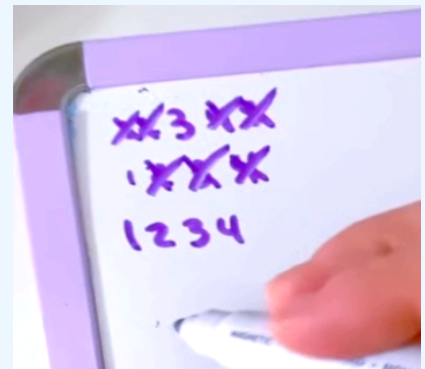
Test Preparation

If you are anxious or have no idea where to start, then you are in the right place!

The **NCLEX-RN** and **NCLEX-PN** is an exam composed of a minimum of 75 questions and a maximum of 145 questions. Each test taker gets five hours to complete this exam.

Please mentally prepare to take the full 145 questions. If you only wish and hope for the 75 questions and go past 75, then your self-confidence can plummet. Remember: If you are still taking the exam past 75 questions that means you still have a great chance of passing.

The exam only comes with an **on-screen calculator**, **dry-erase board**, and **ear plugs**. It does not have a strike-out feature. You are provided a dry-erase board. Use the dry erase board to strike out incorrect answers. *Example on Right.*



You are allowed a **break** at the 2.5 and 3.5 hour mark. **TAKE YOUR BREAKS.** Walk, go the bathroom, splash cold water on your face, take a drink, and **RESET**.

A few hours after the test, use the **PEARSON VUE HACK**. Go back to where you registered for the NCLEX and try to reschedule. Once you put your credit card in, if a message pops up, "Our records indicate that you have recently scheduled this exam. Another registration cannot be made at this time" then that means you passed.

Time for a Pep Talk

I am not your mom, but if I was then I would be so proud of you for getting this far. Most people do not make it to the point where they can take the NCLEX.

If you passed Nursing School, you are **SMART** and **CAPABLE ENOUGH** to pass this exam! Show the State Boards of Nursing that you will be a **SAFE** nurse just like you showed your Nursing School and Clinical Instructors.

If you fail or need to retake this exam, there is no shame in retaking this exam. **Do not be afraid to fail, be afraid to not try.** The benefit of retaking the exam is that you will be more prepared on how to study to pass this exam the next time around.

I did not begin studying for the NCLEX until a month before my exam. Here is exactly what I did to pass in 75 questions:

- 1) I listened to **Mark Klimek** lectures for 1-2 hrs each day. These lectures are available for free on Spotify or Youtube.
- 2) I did 100 NCLEX style questions a day on **UWORLD**. I did 25 in the morning, 50 in the afternoon, and 25 in the evening.
- 3) I reviewed my **Nursing Notes** each night and even made flashcards of the material to review for 5-10 minutes before bed.

YOU WILL PASS THIS TEST!

Next Generation NCLEX 2023

I have good news: I actually was part of the pilot program that took the Next Generation NCLEX and I have some great tips to help you ace this exam too!

The format may have changed, but the information you NEED TO KNOW is still the same.

Format Changes:

Matrix / Grid	A nurse is preparing to administer to Lisinopril to a client. Indicate whether the interventions or conditions are anticipated, nonessential, contraindicated.	Pregnancy High BP	Anticipated, Nonessential, Contraindicated
Extended "Select All That Apply" Question: A nurse is caring for a 32 yr old client experiencing eclampsia. What symptoms would you expect to see? Select all that apply: - Headaches - Edema - Proteinuria - Decreased Reflexes - Hypertension - Hypotension - Seizures	Drag & Drop Question: A nurse is preparing to insert an indwelling Foley catheter. Arrange the following steps in order. Open sterile pack between client's legs Wipe urinary meatus with cleansing solution Lubricate tip of catheter Put on Sterile Gloves Inflate balloon of catheter	Cloze (Drop-Down) "Fill in the Blank" Question: The nurse walks into a patient's room with pneumonia and notices the patient is cyanotic. Complete the following sentences by choosing from list of options. The nurse should first Select.... - Call Provider - Attach Pulse Ox - Apply Oxygen - Auscultate Lungs	Hot Spot Highlighting Question: The oncoming nurse received the following hand off report from night shift. Which statement(s) should prompt the oncoming nurse to ask clarification regarding the patient's status? Highlight the words or phrases to answer. A 79 yr old female client was admitted for a fall. The client is full code and has allergies to compazine and nystatin. The client is AxOx3, independent, full diet. The client has a history of falls, depression, anemia, and COPD. The client's recent vitals were: HR 80, RR 16, BP 122/82, Temp 100.4, Pain 4/10. The client has been having incontinence despite no medical history of this issue. The client is being discharged today at noon.

Lab Values

BLOOD

Hemoglobin "Think Puberty"



Males: **13-18**

Females: **12-16**

Hematocrit "On Average
3x level of Hemoglobin"

Males: **42-50**

Females: **37-47**

3x

WBCs "Kids aged 4-11

always get sick...."

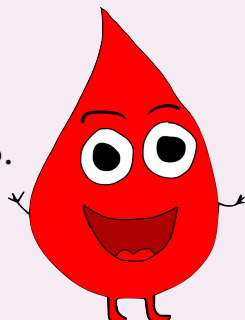


WBC: **4-11K**

RBCs "The word

BLOOD is

5 letters,
between 4-6.



RBC: **4-6**

METABOLISM

Glucose "I got **99** problems
and high blood sugars aint **1**."

Blood Glucose: $99+1 = <100$
A1C: **<5.6**

Cholesterol "It is dangerous
for a car to go over 200 mph"

Cholesterol **<200**

Triglycerides: **<150**



LDL vs. **HDL** 

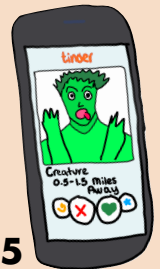
LDL: **<100** L for Lousy Cholesterol

HDL: **>60** It feels good to get High

RENAL

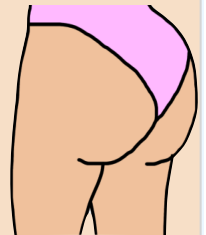
CREATININE "Swipe left.

He is a creatinine
creature and I
rate him 0.5-1.5."



Creatinine: **0.5-1.5**

BUN "If a person
has a nice BUN,
they are a
perfect 10"

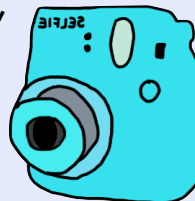


BUN: **10-20**

COAGULATION

PT "Pre-Teens (PT) are
10-12 yrs old"

PT: **10-12**



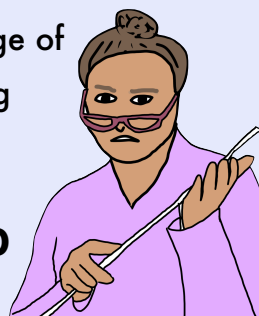
INR "The I in INR looks like
a 1 so **<1.2**"

PT: **<1.2**,
unless taking
warfarin

1NR

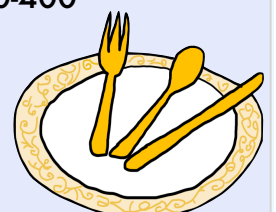
aPTT "A Proper Teaching
Time" aka age of
your Nursing
Instructors.

aPTT: **30-40**



Platelets "A fancy
dinnerware plate set costs
around \$150-400"

Platelets:
150-400K



Electrolyte Values

SODIUM

Sodium Cost of a
Can of Cola

Sodium: **135-145**



MAGNESIUM

Magnesium Magnum Condoms
are needed for men
1.5-2.5x bigger

Magnesium: **1.5-2.5**



CALCIUM

Calcium Call 9-1-1

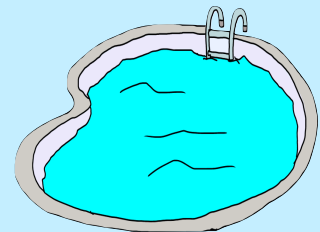
Calcium: **9-11**



CHLORIDE

Chloride It should be 95-105°
to go swimming
in a pool

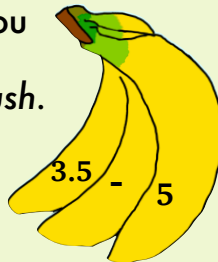
Chloride: **95-105**



POTASSIUM

Potassium # of Bananas you
buy at the store. *Never IV push.*

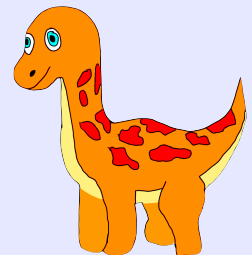
Potassium: **3.5-5**



PHOSPHATE

Phosphate phosphate fossils
are 2.5-4.5
millions years old

Phosphate: **2.5-4.5**



Critical Labs



The Devil's Number is **666**.

6

PH: 6

6

Potassium: 6

6

CO₂: >60

6

O₂: <60

Safety Precautions

STANDARD: stands for all

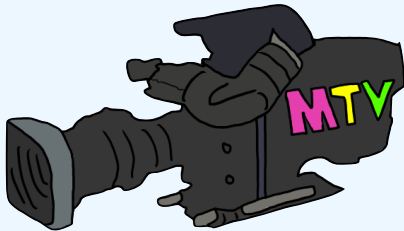
Use: gloves, mask, gown, goggles **as NEEDED**

AIRBORNE: get ready to go on the AIR here at MTV.

M: Measles

T: Tuberculosis

V: Varicella



Use: N95 mask, negative pressure airflow, private room, closed door

Neutropenic Precautions: patient with WBC <1000 or Neutrophils <500

- Strict Hand Hygiene
- No Fresh Fruits or Plants
- Keep Door Closed & No Sick Visitors

DROPLET: drop it like it's hot for your PIMP

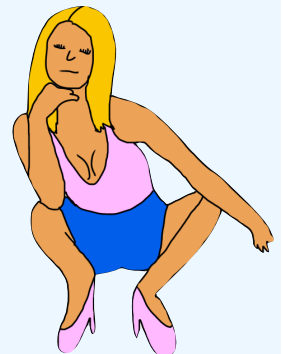
P: Pertussis

I: Influenza

M: Mumps

P: Pneumonia

Use: surgical mask, private room, door closed



CONTACT: do not come into CONTACT with anything in a sewer.

S: Skin Infections

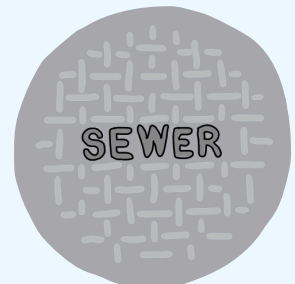
E: Enteric (C. Diff)

W: Wound Infection

E: Eye Infections

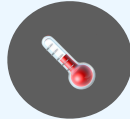
R: Rotavirus

Use: Hand Hygiene, Gloves, Gown, Disposable Equipment



VITALS

BP 120/80



98.6°F (37°C)

O2 95-100

HR 60-100



RR 12-20

Pain 0-10
0: No Pain,
10: Worst Pain

ABGs

NORMAL PH: 7.35-7.45

If **PH: body is irritable**

If **PH: body shutting down**

NORMAL HCO₃: 21-28

HCO₃ is alkaline "basic"

If HCO₃ "Alkalosis": body speeds up =

HR/RR, tremors, irritable

NORMAL PACO₂: 35-45

PACO₂ is acidic

If Acid "Paco₂": body shutting down =

HR/BP/RR, fatigue, coma

CAUSES OF ABNORMAL ABG

Alkalosis: Vomiting, Suction, Hyperventilation

Acidosis: Diarrhea

**UNCOMPENSATED, PARTIALLY
OR FULLY COMPENSATED**

Uncompensated: PH is **ABNORMAL**, PACO₂
OR HCO₃ ABNORMAL

Fully Compensated: PH is **normal**, PACO₂
AND HCO₃ **ABNORMAL**.

Partially Compensated: All three values are
ABNORMAL.

Assistive Devices

Crutches: Move affected extremities & crutches forward first, then unaffected extremities.

Going Upstairs w/ Crutches: Walking up to the **GOOD** place "Heaven": advance **GOOD** leg first, then bring up the bad "broken" leg.

Going Downstairs w/ Crutches: Going down to the **BAD** place: advance the **BAD** extremity down first, then advance the unaffected extremity.

Two Point Gait

RIGHT crutch and
LEFT foot forward

THEN

LEFT crutch and
RIGHT foot

Three Point Gait

Both crutches forward
with weak leg

THEN

Move strong leg
forward

Four Point Gait

Resembles normal
walking

LEFT crutch
RIGHT foot
RIGHT crutch
LEFT foot

Swing Through

Both crutches
forward

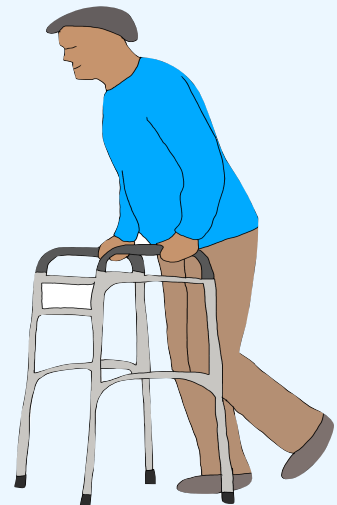
THEN

Both legs swing
forward (one leg
held up)

always move affected (weak) side first

Walker: Client's wrists should be even with hand grips. Advance walker no more than 12 inches forward, advance affected limb forward, and then unaffected limb forward.

Cane: The correct length is measured from the greater trochanter to floor. Cane is used on unaffected (strong) side to provide support to weak side. Advance weak leg first, then strong leg.



Restraints

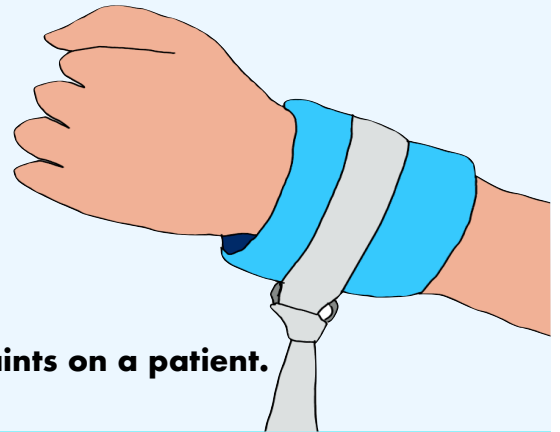
Restraints: Used as a last option only for clients that are at danger to either themselves or others.

Just multiply time X2.

<9 yrs old: **1** hr

9-17 yrs old: **2** hrs

18+ yrs old: **4** hrs



An MD order is needed within 1 hr of applying restraints on a patient.

Drug Abuse & Withdrawal

Alcohol or Drug Abuse: Denial is the #1 problem in all abuse situations.
If it is abuse: confront the problem.
If it is loss or grief: support.

Stages of Grief: Denial, Anger, Bargaining, Depression, Acceptance

Alcohol, Opioids, Cannabis, Benzodiazepines, Barbituates = DEPRESSANT

Intoxication "Too Much" = drowsiness, respiratory depression, slurred speech, unsteady gait

Withdrawal "Too Little" = anxiety, increased HR/BP/RR, tremors, nausea, seizures

Cocaine, Amphetamines, Hallucinogens = STIMULANT

Intoxication "Too Much" = increased HR/BP/RR, Euphoria, Pupil Dilation, Anxiety

Withdrawal "Too Little" = Fatigue, Depression, Suicidal, Exhaustion, Headache

Treatment: Alcohol Withdrawal (Diazepam), Alcohol Abstinence (Disulfiram), Opioid Withdrawal (Methadone), Opioid Overdose (Naloxone), Nicotine Withdrawal

Heart Rhythms

6 Most Tested Rhythms on the **NCLEX**

Lack of QRS = **Asystole**

Saw Tooth Appearance = **Atrial Flutter**

Chaotic = **Atrial Fibrillation** with P Wave

Chaotic = **Ventricular Fibrillation** with QRS Complex
SHOCK

Bizarre = **Ventricular Tachycardia**
SHOCK

Periodic Widened QRS = **PVC**.
Low Priority unless 6+ PVCs in a minute or in a row.

ASYSTOLE

ATRIAL FLUTTER

AFIB

VTACH

VFIB

TREATMENT FOR **ATRIAL** ARRHYTHMIAS

Atrial = P Wave Issue

ABCDE

A: Adenosine

B: Beta Blockers "LOL"

C: Calcium Channel Blockers "PINE"

D: Digitalis

E: Electrocardioversion "AFIB or Flutter"

TREATMENT FOR **VENTRICULAR** ARRHYTHMIAS

Ventricular = QRS Complex Issue

PALS

P: Procainamide

A: Amiodarone

L: Lidocaine

S: Sotalol or Shock for VFIB ⚡

Diabetes

Type 1 Diabetes

Remember your **K's**

K: Keep insulin nearby. Insulin Dependent.

K: Kids. Adolescent Onset.

K: Ketosis. DKA in Type 1

Treatment: Without Insulin, they will DIE!

D: Diet

I: Insulin * * *

E: Exercise

P's of Diabetes: Polyuria, Polydipsia, Polyphagia

Type 2 Diabetes

Remember your **A's**

A: Adult Onset

A: Absent Ketones. No Ketosis.

A: Avoid Excess Carbs/Calories.

Treatment: DREAM!

D: Diet

R: Resources. Patient Education Needed.

E: Exercise * 30 min/5 days a week

A: Additional Insulin if Needed

M: Metformin. Oral antidiabetic med.

Diabetes Insipidus

Decreased ADH causes =

Polyuria (Excessive Urination) +
Polydipsia (Excessive Thirst) =

DEHYDRATION

Insipidus "I need a sip of dis drink"

DKA

Found in Type 1 and caused from acute infection, too much caloric intake or not enough exercise.

Symptoms: **DKA**

D: Dehydration

K: Ketones, Kussmaul Breathing, K⁺ Elevated

A: Acidosis & Acetone Breath

Hypoglycemia: Remember the D's

D: Decreased BG < 70

D: Drunk Sx (slurred speech, delayed reaction, increased HR/RR, decreased BP)

D: Drink Juice. Rapid carbs like Juice or Crackers

D: Drug injection of GLUCAGON.

INSULIN

LEARN the

types of Insulin

L: Long Acting

- Lantus "Glargine"
- Long Acting
- Lasts 24 hrs
- Lack of Peak

(Duration: 12-24 hrs)

E: Rapid Acting

- Eat with injection
- Effective Immediately
- Eliminates a LAG of ↓ BG

L: Lispro

A: Aspart

G: Glulisine

(Onset: 15 min, Peak: 30 min, Duration: 3 hrs)

R: Short Acting

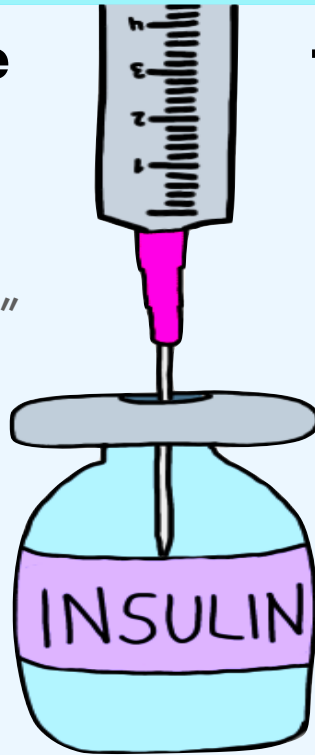
- Regular
- Run IV
- Rapid to Run

(Onset: 1 hr, Peak: 2 hr, Duration: 4 hrs)

N: Intermediate Acting

- NPH
- Not Fast
- Not Clear
- Non-Compliant Patients

(Onset: 6 hrs, Peak 8-10 hrs, Duration: 12 hrs)



A: Adverse Effects of H's: Hypoglycemia, Hypokalemia, Hives, Headache

OB-Maternity

Estimated Due Date:

LMP is last menstrual period

LMP - 3 Months + 7 days

Positive Pregnancy Signs (+)

Fetal Heart Tones or Fetal Skeleton

Weight Gain:

Average Weight Gain for Normal BMI:
30 lbs

Hack: Weeks Pregnant - 9 = Weight Gained in Lbs.

Period: based on a 28 day cycle, fertile 2nd week of cycle.

Trimester

1st Trimester: 1-3 Months

2nd Trimester: 3-6 Months

3rd Trimester: 6-9 Months

Fetal HR: 110-160 "VEAL CHOP"

V: Variable

C: Cord Compression

E: Early

H: Head Compression

A: Accelerations

O: Okay

L: Late

P: Uteroplacental Insufficiency

Stages of Labor

Stage 1: Labor with 3 Phases "LAT"

Latent Phase: dilated 0-3 cm, contraction 5-30 min apart

Active Phase: dilated 4-7 cm, contractions 3-5 min apart

Transition Phase: dilated 8-10 cm, Contraction 2-3 min apart

Stage 2: Birth of Baby.

Stage 3: Birth of Placenta

Stage 4: Postpartum. 2 hrs post delivery.

Postpartum / Newborn Care

MATERNAL CARE

4 things to do in **4th** Stage:

Vital Signs

Fundus: displaced = catheterize, boggy = massage

Assess Perineal Pads with Lochia. Lochia: vaginal discharge.

Reposition & Check for Bleeding

NEWBORN

DRY & WARM BABY IMMEDIATELY

APGAR: tested at 1 & 5 minutes of life of Fetal HR, RR, Muscle Tone, Reflex Irritability, and Color.

Score: 0-3 (severe fetal distress), 4-6 (moderate fetal distress), 7-10 (little-no fetal distress)

Pediatric Milestones

1

1 Month: Grasp Reflex

4

4 Months: Rolls front to back, smiles

6

6 Months: Rolls back to front, first sounds, teeth erupt, solid foods introduced

8

8 Months: Sits Unsupported

12

9-12 Months: Start to Walk

24

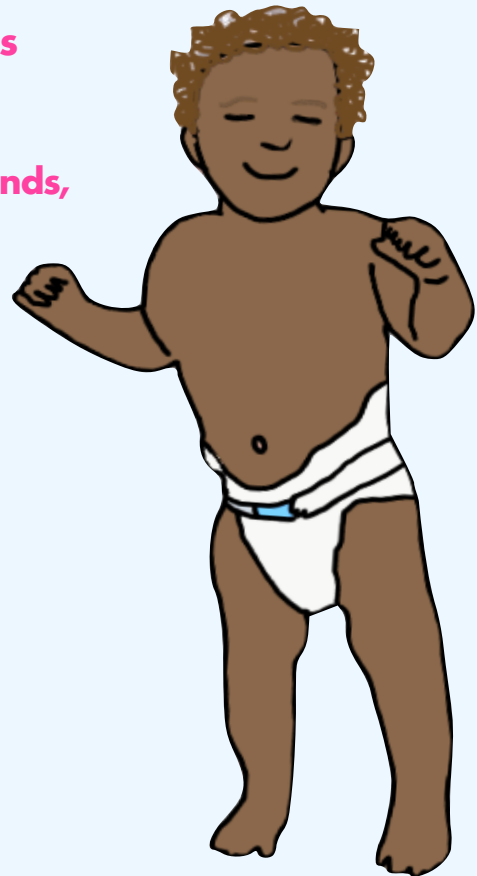
"2 Yrs": Kicks Ball, Walks Up and Down Stairs

36

"3 Yrs": Runs, Draws Circles

48

"4 Yrs": Uses Scissors, Pours Beverage



Nurse Triage

Red: This person was **HIT** hard.

H: Highest Priority

I: Immediate Tx Needed

T: Threatening & Major Injuries

Yellow: Yellow is as bright as the **SUN**.

S: Second Highest Priority

U: Urgently treat within 2 hrs

N: Non life-threatening, but major injuries

Green: These people **WIN** at life.

W: Wait hours-days for tx

I: Injuries Minor

N: Non life-threatening

Black: RIP to the lives we **LOST**.

L: Lowest Priority

O: Only Comfort Measures

S: Survival Chances ↓

T: Tx Contraindicated since pt is pulseless/breathless

Therapeutic Communication

Therapeutic Communication is an exchange of information between at least two people that focuses on the physical and emotional wellbeing of a patient.

Types of Communication: VERBAL and NONVERBAL (body language, facial expression, gesture)

Communication Styles:

Passive: only thinking about others and not self.

Aggressive: focus on own needs and may seem as attacking.

Assertive: thinking about self yet still respecting others.

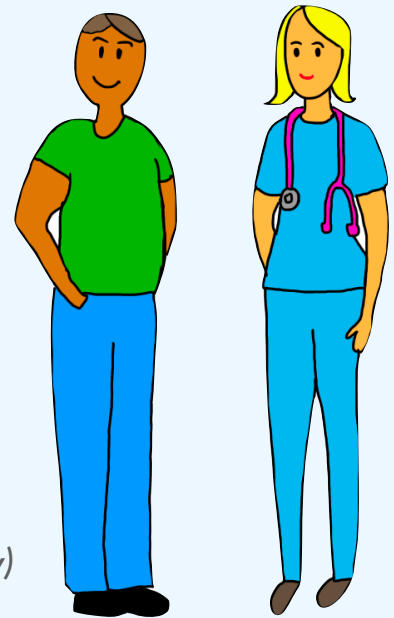
Communication Techniques by Age:

Infants/Toddler: Use Touch

Older Children: Pictures

Adolescents: Verbal

Elderly: Adjust According to Patient (Ex: Hearing or Sight Difficulty)



✓ **WHAT?** "What makes you feel that way?"

✗ **"DON'T WORRY"**

✓ **ACTIVE LISTENING:** Nodding,
Responding Appropriately

✗ **WHY?** "Why are you acting like this?"

✓ **OPEN ENDED ?s:** How do you
feel today?"

✗ **CLOSE-ENDED ?s:** "Do
you have pain?" Usually Yes/No.

✓ **NONVERBAL CUES:** Waving,
Demonstrations

✗ **NOT LISTENING**

Diet Cheat Sheet



CONDITION	DIET
Acute Cholecystitis	NPO to rest gallbladder
Acute Intestinal Obstruction	NPO to rest gallbladder
Kidney Disease	↓ Protein, ↓ Sodium, ↓ Potassium & Fluid Restriction
Liver Cirrhosis	↓ Protein, ↓ Sodium, Fluid Restricted
Constipation	↑ Fiber ↑ Fluid
COPD	↑ Calorie, ↑ Fat, Small Frequent Feeding
Diabetes	↓ Carbohydrates, ↓ Fat, ↑ Fiber
Diarrhea	Liquid, ↓ Fiber, Fluid & Electrolyte Replacement
Gallbladder Disease	↓ Fat, ↑ Protein, Calorie Restricted
Gouty Arthritis & Hyperuricemia	↓ Purine Diet
Hepatitis	↑ Calorie, ↑ Protein
Heart Failure	↓ Sodium, ↓ Fat, Calorie Restricted
Hypertension	Dash Diet: ↓ Fat, ↓ Sodium, ↑ Fiber, ↑ Protein
MAOI Medication Therapy	Tyramine Restriction (smoked meats, cheeses, pickles)
Osteoporosis	↑ Calcium
Pancreatitis	↓ Fat, TPN or Small Frequent Feedings
Peritonitis	NPO to rest gallbladder
Stroke	NPO or Tube Feeding
Vomiting	Fluid & Electrolyte Replacement

NPO: Nothing By Mouth

Full Diet: No Dietary Restrictions

Cardiac Diet: ↓ Sodium, ↓ Fat, ↓ Alcohol

Clear Liquids: Transparent Liquids "water, tea, broth, jello"

Full Liquids: Any Liquids "Ice-Cream, Pudding, Fruit Juice"

TPN: Total Parenteral Nutrition

Pharmacology

Clozapine "APINE": Atypical Antipsychotic
Warfarin "ARIN": Anticoagulant
Losartan "ARTAN": Antihypertensive
Lidocaine "CAINE": Local Anesthetic
Penicillin "CILLIN": Antibiotic
Oxycodone "CODONE": Analgesic
Metronidazole "DAZOLE": Antibiotic
Amlodipine "DIPINE": Antihypertensive
Alendronate "DRONATE": Osteoporosis
Furosemide "IDE": Diuretic
Metoprolol "LOL": Antihypertensive
Fluconazole "NAZOLE": Antifungal
Digoxin "OXIN": Anti-arrhythmic
Naloxone "OXONE": Opioid Overdose
Lorazepam "PAM/LAM": Anxiolytic
Acetaminophen "PHEN": Antipyretic, Analgesic
Epinephrine "PHRINE": Vasopressor
Theophylline "PHYLLINE": Bronchodilator
Omeprazole "PRAZOLE": Antacid PPI
Ibuprofen "PROFEN": Pain, Fever, Inflammation
Ondansetron "SETRON": Nausea
Prednisone "SONE": Anti-inflammatory
Atorvastatin "STATIN": Cholesterol Reducer
Albuterol "TEROL": Bronchodilator
Ranitidine "TIDINE": Antacid
Sumatriptan "TRIPTAN": Migraine
Trazadone "ZODONE": Antidepressant
Alprazolam "ZOLAM": Anxiolytic

⚠️ ANTIDOTES ⚠️

Acetaminophen ➡ Acetylcysteine
Alcohol Withdrawal ➡ Librium
Anticholinergics ➡ Physostigmine
Aspirin ➡ Sodium Bicarbonate
Benzodiazepines ➡ Flumazenil
Beta Blockers ➡ Glucagon
CCB ➡ Calcium, Insulin, Glucagon
Cholinergics ➡ Atropine
Digoxin ➡ Digoxin Immune Fab
Ethylene Glycol ➡ Fomepizole
Heparin ➡ Protamine Sulfate
Insulin ➡ Glucose
Mag Sulfate ➡ Calcium Gluconate
Methanol ➡ Ethanol
Methotrexate ➡ Leucovorin
Opioids ➡ Naloxone
Warfarin ➡ Vitamin K

QUIZLET: Top NCLEX Medications



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<https://quizlet.com/558570870/nclex-prep-pharm-flash-cards/>

How to Answer NCLEX Questions

You do not need to know everything, just know how to look for clues to answer questions!



Prioritize **Acute** vs chronic



For possible intervention, choose **least** invasive first.



Prioritize **UNSTABLE** vs stable
Unstable: acute, post op <12 hrs, newly admitted, newly diagnosed, changed assessment



Avoid extreme answers that have "Always", "Never"



Turn each answer into a **True/False** statement



Do not "Yes" or "No" questions except if **self-harm** involved



If two answers are the same, both are wrong.



Use your **ABC's** (airway, breathing, circulation)



If two answers are opposite, chances are one is right.



If you have never heard of one of the answers, do not pick it.



"What is the nurse's **best** action...?" means what is the one thing that might actually solve the problem.



Only RNs can "**EAT**" (evaluated, assess, teach), LPN/LVN can do reassessments or re-education.



"What is the nurse's **first** action...?" means in a list of steps, what should be done first?



Only delegate tasks to **STABLE** clients with **EXPECTED** outcomes



Assess unless in distress. Assess before any intervention unless it is an emergency.



Use **Maslow's Hierarchy**, physical needs over psychosocial

Rights of Delegation: "Terrific Care Promotes Client Safety"

RIGHT Task, Circumstance, Person, Communication, Supervision

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You WILL pass! I believe in you!
Xo Amanda